

Addendum:

1. **Above the peritoneal reflection within the pelvis:** Disease is absent and Ureters are free of tumour
Below the Peritoneum anteriorly: Prostate is not infiltrated by tumour but <1mm clearance
2. **Posteriorly: no tumour**
Sciatic nerve/ S1/S2 nerve roots: No tumour
3. **Laterally:** Right pelvic sidewall compartment is infiltrated by mucin signal tumour
Internal/external iliac arterial /venous branches and Sacrotuberous/sacrospinous and Piriformis/Obturator are free of tumour
4. **Infralevator compartment:** Levator muscles are infiltrated on the right Sphincter complex is free of tumour
5. **Anterior urogenital triangle/Perineum:** Retropubic space: free of tumour

Opinion:

y MRI Overall stage: T3c N1, TRG3., Poor prognosis, TME CRM pos and EMVI positive

Total number of compartments 4 (central, right lateral, infralevator and anterior below the peritoneal reflection). Closest potential surgical margins are located r lateral PSW and prostate anteriorly

Resection would require exenterative resection with en bloc removal of prostate/seminal vesicles, extended right lateral dissection beyond TME

Addendum Signed By : Gina Brown

Addendum Date: 17/05/2016

Imported external MR for MDT meeting IEP**Clinical information:****Findings**

The primary tumour is demonstrated as an semi-annular mucinous mass with a infiltrating border.

The distal edge of the luminal tumour arises at a height of 48mm from anal verge.

The distal edge of the tumour lies 8mm above the top of the puborectalis sling.

The tumour extends craniocaudally over a distance of 66mm.

The proximal edge of tumour lies below the peritoneal reflection.

The invading edge of tumour extends from 11 to 1 o'clock.

Tumour is extends through the muscularis propria.

Extramural spread is 10mm.

MR T stage: T3c.

Tumour is not present at the distal levator level.

Additional comments:**Lymph nodes assessment:**

<3 mixed signal/irregular border and mucinous component N1

Extramural venous invasion:

No evidence

CRM:

Closest circumferential resection margin is at 12 o'clock.